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Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

PRESIDENTIAL EBOLA TASK FORCE 17

MVE17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°155 /MVE-BDBV/16/10/2026

Report date: October 16, 2026 Publication date: October 17, 2026

HIGHLIGHTS (last 24 hours)

- **As of September 16, 2026, a new province was affected, namely North Ubangi, specifically the Wapinda health zone. Sixty-seven (67) new confirmed cases of the Ebola Virus Disease (EVD) cases, including 18 community deaths, have been reported. These cases originated from provinces of Ituri (44 cases), North Kivu (14 cases), Haut-Uélé (7 cases) and North Ubangi (2 cases), In addition, 8 deaths were recorded in the CTEs.**

ACCUMULATIVE CAS	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
9415	4 536	48,2 %	916	2 522	79,1%

Affected provinces and affected health zones

8 Provinces affected

Uele, Ituri, North Kivu, South Kivu, Tshopo, Lower Uele and Sud Ubangi

66 Health Zones affected out of 178(37,1%)

Summary of key points from the last 24 hours

Over the past 24 hours, 67 new confirmed cases of Ebola virus disease (EVD) have been reported in four provinces: **Ituri (44 cases),** North Kivu **(14 cases), Haut-Uélé (7 cases), and North Ubangi (2 cases),** which becomes the eighth province affected by the outbreak with the Wapinda health zone. Among these cases, **18 community deaths** have been reported. In addition, **8 confirmed deaths** were recorded in Ebola Treatment Centers (ETCs), **bringing the total number of confirmed deaths to 26.**

notified during the period

Furthermore, **15 patients (14 in Ituri and 1 in Tshopo)** were declared cured after obtaining two negative results in the control test for Ebola Virus Disease.

A new province has been affected: North Ubangi, specifically the Wapinda health zone, which has reported two confirmed cases, including one death. The two individuals are men in their twenties, residing in the Wapinda health zone, which borders the Bas-Uélé province.

Since the start of the epidemic, the DRC has accumulated **9,415 confirmed cases and 4,536 deaths, representing a case fatality rate of 48.2%.** Ituri remains the epicenter of the epidemic, with **73.2%** of cumulative confirmed cases and **65.7%** of new confirmed cases reported in the last 24 hours.

66 out of 178 health zones (37.1%), spread across eight provinces, have been affected since the start of the epidemic.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of October 16, 2026.

Province	New cases confirmed (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	44	6891	3 180	46,1%	28/36 (77,8 %)
North Kivu	14	2027	1 150	56,7%	17/34 (50,0 %)
Haut-Uélé	7	406	174	42,9%	7/13 (53,8 %)
Boss	0	71	25	35,2%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	0	13	4	30,8%	4/11 (36,4%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Nord Ubangi	2	2	1	50,0%	1/11 (9,1%)
Total	**67**	**9 415**	**4 536**	**48,2%**	**66/178 (37,1%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 16, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**6891**	**3180**	**46,1%**	**44**	**10**	**6**	**16**
Give	11	1	9,1%				0
Understand	10	6	60,0%	1	0	0	0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	216	48	22,2%				0
have	2	2	100,0%				0
Bunia	1877	625	33,3%	8	2	0	2
Ladies	40	9	22,5%				0
Drodro	15	7	46,7%				0
Fireworks	84	31	36,9%				0
Gethy	11	7	63,6%				0
Flounder	2	0	0,0%				0
Kilo	38	12	31,6%				0
Team	188	113	60,1%	1	0	0	0
Liter	308	156	50,6%	1	0	1	1
Logo	14	8	57,1%				0
Fight	36	9	25,0%				0
distributed	4	1	25,0%				0
Washing	68	28	41,2%	3	1	2	3
Darkness	316	156	49,4%	18	7	0	7
Mangala	379	162	42,7%	4	0	0	0
Mongbwalu	713	342	48,0%				0

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Nia -Nia	329	190	57,8%				0
Nizi	824	298	36,2%	2	0	1	1
Legumes	134	39	29,1%				0
Jungle	19	5	26,3%				0
Rwampara	1157	400	34,6%	6	0	2	2
Tchomia	76	25	32,9%				0
A valves	THAT	493	THAT				0
North Kivu	**2027**	**1150**	**56,7%**	**14**	**4**	**0**	**4**
Me	502	339	67,5%	4	3		3
Farm	23	7	30,4%				0
Butembo	311	187	60,1%				0
Ten	1	0	0,0%				0
Kalunguta	76	29	38,2%	1	0		0
Cut	639	391	61,2%	2	1		1
Source	3	2	66,7%				0
Kyondo	44	21	47,7%	2	0		0
Lubero	35	14	40,0%	3	0		0
Be careful.	70	28	40,0%				0
Alimbong	5	3	60,0%				0
Manguredjipa	15	5	33,3%				0
Masereka	39	13	33,3%				0
The museums	151	71	47,0%				0
Mutwanga	24	5	20,8%				0
Oicha	49	21	42,9%	2	0		0
Vuh	40	14	35,0%				0
Upper Uele	**406**	**174**	**42,9%**	**7**	**3**	**2**	**5**
Government Mangbetu	45	19	42,2%	1	0	0	0
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	105	46	43,8%				0
Lining	102	53	52,0%	0	0	2	2
Beans	5	3	60,0%				0
Ordinary	145	51	35,2%	6	3	0	3
Boss	**71**	**25**	**35,2%**	0	0	0	0
Bafwasende	17	2	11,8%				0
Kabondo	8	1	12,5%				0
Lubunga	1	0	0,0%				0
Maquis - Kisangani	32	14	43,8%				0
Mangobo	10	7	70,0%				0
Boss	2	0	0,0%				0
Wanie - Arugula	1	1	100,0%				0
South Kivu	**3**	**1**	**33,3%**	0	0	0	0
Miti -Murhesa	3	1	33,3%				0
Bas Uélé	**13**	**4**	**30,8%**	0	0	0	0

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Blind	1	1	100,0%				0
Viadana	6	1	16,7%				0
Claim	1	0	0,0%				0
Walk	5	2	40,0%				0
Sud Ubangi	**2**	**1**	**50,0%**	0	0	0	0
Hair	2	1	50,0%				0
Nord Ubangi	**2**	**1**	**50,0%**	2	1	0	1
You are logged in.	2	1	50,0%	2	1	0	1
Total	**9415**	**4536**	**48,2%**	**67**	**18**	**8**	**26**

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA : Not Applicable.

Table 3. Status of alerts notified as of October 16, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	132	2	134	13	2	91	0	15	11
Bas Uélé	133	3	136	14	3	119	0	17	0
Ituri	1 105	94	1 199	135	94	967	0	212	107
North Kivu	1 592	67	1 659	191	66	1 401	1	257	153
Sud Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
Boss	189	3	192	9	3	180	0	12	0
Total General	**3 151**	**169**	**3 320**	**362**	**168**	**2 758**	**1**	**513**	**271**

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

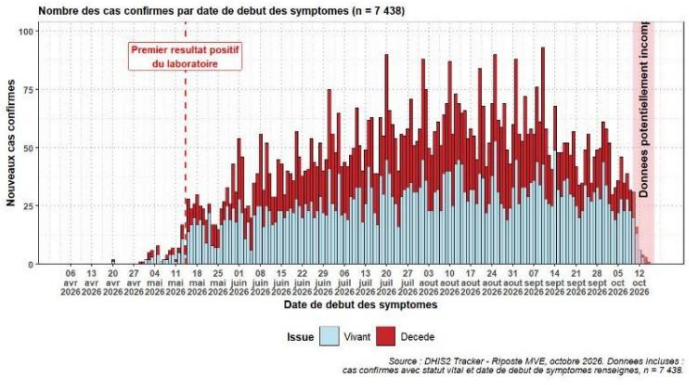
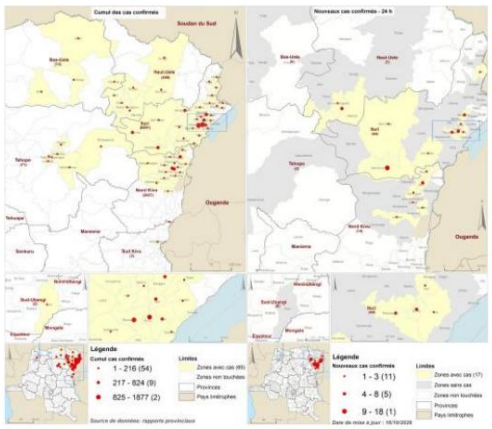


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



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1.COORDINATION

1.1. Main Actions

- Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

- On October 16, 2026, 3,320 alerts were reported in the five provinces that submitted their data: North Kivu, Ituri, Haut-Uélé, Bas-Uélé, and Tshopo. Of these alerts, 3,289 (99.1%) were verified, with 530 (16.1%) validated as suspected cases and 2,759 (83.9%) invalidated. A total of 513 suspected cases (96.8%) were investigated, and 271 (51.1%) were transferred to treatment centers/infection control centers. Furthermore, 31 alerts (0.9%) remained to be verified.

- Among: Of the 33,125 contacts to be followed up in the six provinces that submitted their data, 26,216 were seen in the last 24 hours, representing a follow-up rate of 79.1%, slightly down from the previous day (79.2%) and below the required threshold of 95.0%. This rate was 100.0% (44/44) in South Ubangi, 92.7% (745/804) in Bas-Uélé, 89.6% (1,365/1,523) in Haut-Uélé, 84.1% (14,381/17,105) in North Kivu, 81.4% (978/1,202) in Tshopo, and 69.9% (8,703/12,447) in Ituri, where monitoring remains particularly low in Bunia (24.9%; 906/3,635) and Nia-Nia (57.1%; 220/385); in Haut-Uélé, it is only 64.9% (122/188) in Boma Mangbetu.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

- On October 15 and 16, four alerts were notified to the PoE/PoC in the five monitored provinces: two in Ituri and two in South Kivu. These included two live alerts validated as suspected cases in Ituri (Biakato Mayi and Foner Mambasa PoCs), one of which (50.0%) was transferred to the CT/CTE, the second suspect having refused transfer, and two lifeless bodies intercepted in South Kivu (Kavimvira and Kivovo Port PoEs), all swabs (100.0%); all live alerts were investigated.

Table 4. Status of key PoE/PoC indicators by province, as of October 16, 2026

Indicators	Ituri	North Kivu	South Kivu	Boss	Haut-Uélé	Global
Report completeness	98,4% (60/61)	85,7% (18/21)	55,6% (5/9)	100,0% (18/18)	100,0% (10/10)	93,3% (111/119)
% of functional strategic PoE/PoC	100%	100%	ND	100%	90%	ND
Name of people switched to PoE/PoC	144 661	79 704	8 850	18 858	23 425	275 498
% of travelers screened	98,2%	98,0%	98,7%	98,9%	99,5%	98,3%
% of travelers who washed their hands	97,5%	98,0%	98,7%	98,2%	99,3%	97,9%
% of travelers made aware	99,7%	98,0%	100,0%	100,0%	100,0%	99,3%
Number of alerts notified	2	0	2	0	0	4
Number of live alerts validated (suspected cases)	2	0	0	0	0	2
Number of bodies recovered today	0	0	2	0	0	2
% of suspected cases transferred to the CTE/CT	50,0%	THAT	THAT	THAT	THAT	50,0%
% of lifeless bodies swabed/secured	THAT	THAT	100,0%	THAT	THAT	100,0%
% of suspected cases investigated within 4 h	100,0%	THAT	THAT	THAT	THAT	100,0%

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2.2.2. Challenges

- **In Ituri,** insufficient mobility for supervision in the health zones of Rwampara, Nizi, and Bunia, and the delayed payment of service providers for September, persist; the Avakubi Point of Contact (PoC) remains non-functional due to insecurity. In North Kivu, data completeness has decreased to 85.7% (18/21), as the Kyanghala and Cugeki PoCs have not transmitted their data, the Butuhe PoC remains closed since the security incident of September 26, 2026, the security threat persists against service providers along the Butembo axis, alerts raised by the PoCs are not being addressed in a timely manner, and the PoCs lack consumables, communication equipment, and a dedicated ambulance. In South Kivu, the completeness rate remains at 55.6% (5/9), the antennas at the Baraka-Fizi station are not reporting, and the PoEs at Kavimvira, Kalundu Port SNCC, and Baraka lack isolation devices. In Haut-Uélé, the activation of the PK18 and Bangwene PoCs in the Wamba health zone is still underway. In Bas-Uélé, only 20 out of 41 PoEs/PoCs (48.8%) are functional, PNHF services are not operational, and the poorly trained and demotivated service providers lack water, transportation, connectivity, and visibility badges.

2.3. Laboratory

2.3.1. Main actions

- **Ituri:** 44 new positive results (34 living and 10 dead) out of 155 samples received and analyzed (positivity: 28.4%).

- **North Kivu :** 14 new positive results (10 living and 4 dead) out of 239 samples received and analyzed (positivity: 5.9%).

- **Haut-Uélé :** 7 new positive results (4 living and 3 deceased) out of 28 samples received and analyzed (positivity: 25%).

- **Tshopo :** 7 new samples received at the Buta and Isiro laboratory, all analyzed negative.

- **Bas Uélé :** 26 new samples received at the Buta and Isiro laboratory, all analyzed negative.

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri :** 13 out of 41 rings were opened (31.7%), 40 out of 42 households were decontaminated (95.2%), and 16 out of 17 health facilities were decontaminated (94.1%). Of 99 EDS alerts (including 14 postponements from the previous day), 59 EDS were carried out (59.6%), and 75 out of 92 bodies were swabed (81.5%).

- **Tshopo :** 13 ESS (Emergency Situations) were monitored in 5 health zones and 182 healthcare providers were briefed on standard precautions, triage, and screening; IPC (Infection Prevention and Control) scores were reassessed at Alabul Hospital (40.3%) and Lokosa Health Center (64.5%), and the ISTA (Institute of Tropical Medicine) and the CEDEC (Center for the Study and Coordination of Education) building were decontaminated around the 59th confirmed case (Makiso-Kisangani); 4 bodies were swabed and 2 EDS (Emergency Discharge Summary) tests were carried out

- **Haut-Uélé :** 4 rings were opened for 7 confirmed cases of the day (57.1%), 8 out of 8 households and 2 out of 2 ESS were decontaminated in Wamba and Pawa, 48 PPL were briefed (25 in Isiro, 11 in Boma Mangbetu, 7 in Wamba and 5 in Pawa) and the PCI scores of the ESS evaluated range from 21% at CS Basobuo (Pawa) to 84% at the HGR of Wamba; the 4 EDS alerts resulted in 4 EDS (100%) and 3 bodies were swabed, the only body not swabed being that of a confirmed case who died at the CTE.

- **North Kivu :** 15 rings were opened out of 19 (78.9%), 10 households decontaminated out of 18 (55.6%) and 10 out of 10 ESS (100%); 13 ESS were assessed (average PCI score of 35.5%) and the risk of exposure of 37 providers was assessed, including 2 high-risk PPLs, with no infected PPLs among the last 22 confirmed cases.
  Of 104 EDS alerts (including 45 reported from the previous day), 60 EDS were carried out (57.7%) and 59 bodies were swabed; the training of 7 ECUMR teams began in the Mabalako health zone with the support of WHO and FHI360.

- **Bas-Uélé :** Uélé: 11 community health workers were monitored in the health zones of Buta (3/3), Viadana (3/3), and Ganga (5/5), and 36 workers and 32 hygiene professionals were briefed on the use of PPE, waste management, and sorting (100%); the DLM (Digital Medical Laboratory) is operational at the Buta General Referral Hospital (13/13), the Triangle Health Center (3/3), and the Mobenge Health Center (3/3); the Kavio Health Center achieved an IPC (Infection Prevention and Control) score of 17%; and the Kavio, Nkumu Nyele, and Mazulu (Viadana) Health Centers received PPE and IPC supplies from the WHO. Three swabs (swabbing) were conducted (2 in Ganga and 1 in Koteli), without a DHS (Digital Health Survey).

2.4.2. Challenges

- **In Ituri,** the opening of the rings remains very insufficient (13/41; 31.7%), 31 EDS failed and 9 were postponed to the next day, and 17 bodies out of 92 (18.5%) could not be swabed.

- **In North Kivu,** community resistance persists in Katwa, Beni, Butembo, and Kalunguta; decontamination teams remain insufficient in Katwa, Biena, Masereka, Kyondo, Mabalako, and Vuhovi; none of the 10 decontaminated households received kits (0/10); 44 EDS (Emergency Health Distribution) operations remain postponed; ECUMR (Emergency Community Medical Response) coverage is insufficient; and the road to the Bundji cemetery (Beni) is impassable. In Tshopo, IPC (Infection Prevention and Control) inputs and materials are lacking in health establishments and communities; there remains a shortage of frontline providers trained in IPC for Ebola Virus Disease; and the absence of families hinders the implementation of swabs (healthcare swabs). In Haut-Uélé, only 4 ZS (Zones de Santé - Health Zones) are operational.

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Of the 7 affected areas, 16 out of 20 have submitted their infection prevention and control (IPC) data; none of the 8 decontaminated households received kits, and IPC kits are lacking in the health centers. In Bas-Uélé, 16 out of 20 priority health centers do not have functional triage facilities, 17 out of 20 do not have a 72-hour water storage capacity, and 20 out of 20 do not have mattresses; one household and one health center in Dakwa remain to be decontaminated due to a lack of equipment, and the low reporting of community death alerts is limiting the health assessments.

2.5. Holistic care

2.5.1. Main actions

- **In Ituri,** 72 new admissions were recorded, compared to 76 discharges: 14 recoveries, 50 non-cases, 11 deaths (6 confirmed), and 1 discharge without medical advice in Lita. Thus, 380 patients (139 confirmed and 241 suspected) are hospitalized, representing an occupancy rate of 37.0% (48.0% for suspected and 26.0% for confirmed cases), including 47 critically ill patients; 21 patients were evacuated by ambulance, and 363 out of 371 patients (97.8%) received a hot meal.

- In North Kivu, 362 patients, 545 people with AIDS (PLAs), and 190 caregivers received hot meals; 16 out of 16 children at the Ebola Treatment Center (ETC) and 2 out of 2 at the daycare center are receiving low-dose/low-dose medication; 7 out of 7 confirmed unstable patients received a specific diet; and 63 out of 63 patient households received food assistance. The Musienene ETC began treating patients on October 16.

- **In Haut-Uélé,** 15 new admissions were recorded against 15 discharges, including 1 recovery. Thus, 44 patients are hospitalized out of 169 beds set up, representing an occupancy rate of 26.0%, including 4 patients in serious or critical condition (2 confirmed in Boma Mangbetu and 2 suspected in Pawa); 11 patients were transported by ambulance and 44 patients, 57 accompanying persons and 70 PPL received hot meals.

- **In Tshopo,** 2 new admissions of suspected cases were recorded, compared to 1 discharge (recovery), with no deaths at the Ebola Treatment Center (ETC). Thus, 18 patients (11 confirmed and 7 suspected) are hospitalized out of 36 beds, representing an occupancy rate of 50.0%. Since the beginning of the outbreak, 42 confirmed cases have received the experimental treatment and 457 people have received post-exposure prophylaxis.

- **In Bas-Uélé,** no new confirmed cases have been recorded; 3 patients remain in isolation, without recovery
no death reported.

2.5.2. Challenges

- In Ituri, the low capacity of some CTE/CT (Nyankunde, Kilo, Lolwa, Mangala, Jina and Biakato), the care of 47 critical patients, the risk of blood stock shortage in the coming days, the lack of food for 8 patients in the Mandima Health Zone (CT Mandima and CT Makeke) and the low reporting of ambulance movements (13/33) remain worrying; the construction of a 60-bed CTE in Biakato is considered urgent.

- In North Kivu, the lack of data on patient movements, bed occupancy and ambulances for October 16 makes it impossible to assess the pressure on CTE/CTs, whose beds for suspected cases were 95.6% full the day before.

- In Haut-Uélé, communication with the CTEs remains difficult due to the lack of Starlink connection in all sites, some staff are demotivated due to lack of payment, nutritional inputs (MUAC, toises, F75, F100, Plumpy'Nut) are lacking and the supply of water and energy remains insufficient in some CTEs.

- In Tshopo, the lack of food and nutritional support at the Bafwasende CT, the 7 confirmed cases lost to follow-up, the lack of resuscitation equipment and the absence of specialists at the CTE weaken the management.

- **In Bas-Uélé,** the lack of an ambulance for transferring patients and of a standardized CTE, still under construction at the HGR of Buta, as well as the lack of computer kits, remain major operational challenges.

2.6. Vaccination

2.6.1. Main actions

- **In Ituri,** a technical meeting to harmonize the lists of staff recruited for the TUMAINI project (clinical trials of a specific candidate vaccine and Ervebo) was held and the collection of data for the study on the impact of EVD on the use of sexual and reproductive health services continues; the cumulative number of vaccinated PPLs remains at 1,120, as the day's data has not yet been transmitted.

- **In North Kivu,** vaccination of people living with HIV (PLHIV) against Ebola virus disease (EVD) was officially launched on October 16, 2026, at the Beni General Referral Hospital (HGR), in the presence of the health coordinator representing the governor, the incident manager of the Presidential Task Force, and partners. Sixty-two PLHIV were vaccinated on the first day in the Beni health zone, out of a target of 7,105 (0.9%), for a provincial target of 36,070 PLHIV in 15 health zones. The province received 600 doses of Ervebo vaccine and 600 syringes, as well as two ultra-low temperature freezers (-90°C to -60°C) and two freezers (-30°C to -15°C), and a vaccination team was identified and briefed. In Haut-Uélé, vaccination of PLHIV was launched in the Bolebole health zone (Wamba), bringing the total number of PLHIV vaccinated to 588.

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Ervebo: 469 in Isiro (190 in Tely and 279 in Mayogo), 80 in Pawa and 39 in Wamba; 489 doses were used out of 600 received.

2.6.2. Challenges

- **In Ituri,** the target for PPL and TPL remains to be redefined for the calculation of vaccination coverage (1,120 vaccinated) for a provisional target of 1,000).

- **In Bas-Uélé,** the vaccination units are not operational, vaccines are not available in sufficient quantities, and vaccination funds are still pending; in Tshopo, a discrepancy of 551 vaccinated individuals persists between the Excel and DHIS2 databases, and the Bafwasende health zone has not reported any vaccinations for four days; in North Kivu, the vaccination team lacks transportation (2 vehicles) and 3 small tents, and the vaccine transport devices (ARKTEK) are still pending confirmation with the WHO.

2.7. Continuity of Care

2.7.1. Main actions • In North Kivu,
the baseline assessment of the Oicha General Referral Hospital (HGR) revealed a triage system that did not comply with the security protocol and was only functional until 5 p.m., the absence of PPE, pedal bins and hygiene products, an unsecured Ebola circuit in the laboratory, deliveries of suspected cases carried out in the common room and the absence of a follow-up plan for TB and HIV patients lost to follow-up; at the Kyondo General Referral Hospital (HGR), the triage system is not compliant, 13 out of 33 maternity staff are not trained in osteoarthritis and prenatal care, only 25 out of 205 staff (12.2%) are trained in infection prevention and control (IPC) and 50 staff are exposed, including 5 in quarantine.

- **In Tshopo,** the assessment of the Central Prison CP found 121 cases of malnutrition in adults and 10 PLHIV being monitored, a shortage of antituberculosis drugs for a cohort of 16 patients, very poor waste management, the absence of a triage area and no staff trained or vaccinated against Ebola (0/10); recovery plans for the Alwaleed CS and the Ketele military PS have been developed and follow-up of recovered patients with complications continues at the Makiso-Kisangani General Referral Hospital.

- **In Haut-Uélé,** the integration of catch-up vaccination of zero-dose and under-vaccinated children and of surveillance in monitoring the distribution of LLINs was advocated in the 13 health zones, the training of 7 health zone managers on SIMR continues and 5 PPLs remain being treated in the CTEs (2 in Wamba, 2 in Pawa and 1 in Boma Mangbetu); the deficit of subsidies for free care persists in the 7 most affected health zones.

- **In Ituri,** 79 children received nutritional support from LNPE/SLM: 9 out of 9 at the CTE, 6 out of 6 at the nursery and 64 out of 64 in the community; AMMA, distribution of MILD and space emanators were launched and medicines for free care were distributed in the targeted ZS.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- **In Ituri,** 20,153 home visits reached 77,251 people; 341 educational talks brought together 6,394 people, 4 mass awareness campaigns reached 603 people, 6 community debates involved 188 people, and 6 advocacy sessions were held with 26 religious authorities and community-based organizations in Rwampara and Komanda. A total of 314 community alerts, including 70 deaths, were reported. 33 out of 46 feedbacks (71.7%) and 5 out of 8 incidents (62.5%) were resolved within 48 hours, and the CREC facilitated the pre-listing of 329 contacts around confirmed cases, the decontamination of 16 out of 16 households (100%), 2 structures and 5 ESS, 13 out of 13 EDS (100%), the announcement of 9 out of 9 positive results and the transfer of 26 out of 26 suspected cases (100%) to CTE; in Komanda, the MCZ held a working session with the chiefs of the Walese Vonkutu and Mungamba chiefdoms to strengthen their involvement around confirmed cases.

- **In Bas-Uélé,** 675 home visits reached 1,569 people in the Ganga health zone, 5 mass awareness sessions reached 1,474 people (Ganga, Poko, Monga and Titule), including 167 worshippers at the Dingila mosque, and an educational talk reached 8 people; 57 people were made aware of the importance of EDS in Ganga, advocacy was carried out with the priest of the Amadi parish (Poko) and a radio program was broadcast in Bondo.

- **In North Kivu,** 6,644 home visits reached 19,419 people, 185 educational talks reached 3,673 people, 45 mass awareness campaigns reached 7,967 people, and one briefing reached 26 people; 266 community alerts were notified out of 650 expected (40.9%), 14 out of 17 feedback requests (82.4%) were resolved within 48 hours, and 851 out of 4,750 people (17.9%) were engaged in community actions, with only 7 out of 14 health zones (50.0%) reporting. The CREC supported the official launch of vaccination for people living with HIV and the start of treatment at the Musienene Ebola Treatment Center, briefed 15 school leaders in Beni, supported the Health Depletion Strategy (HDS) and the decontamination of a household in Lubero, and facilitated the listing of 19 contacts in Manguredjipa.

- **In Tshopo,** doctors in charge of private health centers were made aware of and engaged in Ebola prevention and awareness campaigns, the SGI meetings in 5 health zones were supported, and educational talks were held with the Bomoko association. and the discharge of a new patient from the Météo CTE was accompanied by testimonials; approximately 14,592

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5,292 people (5,292 men, 6,013 women and 3,287 children) were sensitized, 25 community actors were trained in community mental health with support from Africa CDC and the only identified refusal was broken (1/1).

- **In Haut-Uélé,** 95 home visits reached 570 people, 30 educational talks reached 315 people, and 2 mass awareness campaigns reached 834 people in Wamba and Isiro. Advocacy efforts were also conducted with coordinators of Catholic and Protestant schools. Eleven community alerts (9 in Wamba and 2 in Isiro) and two deaths were reported, and two out of three feedback requests were resolved within 48 hours. CREC facilitated the launch of vaccinations for people living with HIV in the Bolebole health zone (Wamba) and, in collaboration with the Continuity of Care pillar, developed messaging integrating the distribution of IBDs, routine immunizations, and the Ebola response.

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- **In Ituri,** 15 of the last 41 confirmed cases (36.6%), 92 confirmed cases under follow-up out of 139 (66.2%), 32 new suspected cases out of 72 (44.4%), and 149 suspected cases under follow-up out of 241 (61.8%) benefited from SMSPS interventions. The teams supported 323 caregivers and 59 people living with HIV, monitored 26 out of 29 recoveries (89.7%) in the community and 14 out of 14 children in daycare, assisted 22 out of 26 separated and orphaned families, reintegrated 12 out of 14 recoveries, facilitated family reunification, and announced 83 results, including 15 positive ones; 119 children are currently being monitored.

- **In North Kivu,** 20 of the last 22 confirmed cases (90.9%), 212 confirmed cases under follow-up, 318 new suspected cases and 215 suspected cases under follow-up benefited from SMSPS interventions, with 6 out of 7 affected health zones covered (85.7%); the teams accompanied 88 households and 32 households of separated and orphaned children out of 50 (64.0%), supported 67 PPLs, followed up 5 out of 5 children in daycare and 102 children in total, reintegrated 13 out of 13 recovered patients, followed up 10 out of 10 recovered patients in the community and announced 96 results, including 22 positive ones; 302 people (143 men, 125 women and 34 children) benefited from psychoeducation and community leaders and APAs were trained in SMSPS with the support of Africa CDC.

- **In Haut-Uélé,** only Isiro and Pawa reported: 1 new confirmed case, 10 confirmed cases under follow-up, 4 new suspects and 14 suspects under follow-up benefited from SMSPS interventions; 36 caregivers and 40 PPLs were supported, 4 separated children were followed in the community, 2 EDS were supported and 13 results announced, including 1 positive; 5 non-cases were discharged from the Pawa CTE and a 30-month-old suspected child was admitted to the Isiro CTE.

2.9.2. Challenges

- **In Ituri,** only 14 of the 28 affected health zones are covered (50.0%), data from the Mandima health zone, which concentrates 51% of the cases, is still not coming in and only 15 of the last 41 confirmed cases (36.6%) have been taken care of; training of psychologists and APS/TPS is scheduled for October 18 and 19, 2026.

- **In Bas-Uélé,** SMSPS coverage remains embryonic, with activities having only just begun in the ZS from Viadana.

- **In Haut-Uélé,** data from Boma Mangbetu and Wamba have not been collected, the mobility of APS remains difficult in these two ZS (6 motorcycles and fuel are needed), nutritional kits are lacking for weaned infants in Pawa and Wamba and the pillar does not have computer equipment.

- **In Tshopo,** the APS of the Wanie-Rukula ZS still need to be trained, 2 out of 7 ZSs that have notified cases are not covered and the pillar lacks communication units and motorcycles for its travels.

2.10. Logistics

2.10.1. Main actions.

- Ensure the mobility of all field teams in the affected provinces; in North Kivu, construction work on the huts at PoC PK5 and the hut and passenger walkway at Beni airport continues with IOM support; in Haut-Uélé, a team went down to Wamba for the briefing of service providers and the activation of PoC PK18 and Bangwene, the maintenance of an ambulance in Boma Mangbetu was monitored, 2 coordination vehicles were supplied with fuel for the mission in Bolebole and response inputs were delivered to the PoE/PoC pillar; In Bas-Uélé, the construction of the CTE at the HGR in Buta (partitioning of rooms and toilets) and the rehabilitation of the DPS offices with the support of the WHO are continuing, the surveillance teams have been deployed in Ganga and Viadana, 2 circuits of hand washing devices have been installed at the PoC PK4 Left Bank with a month's supply of water (support from FHI360) and the inputs for the ZS in preparation are being packaged.

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2.11. Security

2.12. Main actions

- **In North Kivu,** security coverage was provided during the official launch of the vaccination of PPL at the Beni General Referral Hospital by the Military Governor, for the burial of 2 confirmed cases at the Bundji public cemetery, the decontamination of 3 households in the Mangote Health Area and the Saint Jean Hospital and the swab of a lifeless body; the threat persists against the providers of the PoCs on the Butembo axis and the Butuhe PoC remains closed since the incident of 26/09/2026.

- **In Tshopo,** community tensions and a refusal of swabs were reported in Bafwasende, where attacks and threats against rapid response teams are hindering investigations into cases 65 to 70. In Haut-Uélé, the Wamba sub-coordination was relocated to the Bolebole-Mambati resistance zone, and teams were denied access to the Gosene Revival Church (Isiro), where alerts had been reported; in Ituri, the Avakubi PoC remains non-functional due to insecurity

2.13. PSEA (Prevention of Sexual Exploitation and Abuse)

2.13.1. Main actions

- **In Ituri,** 25 IMC staff members (5 women and 20 men) were sensitized on zero tolerance for EAS/HS in the Bunia health zone, 39 new IMC and Samaritan's Purse staff (18 women and 21 men) and 15 community leaders from Mongbwalu (9 women and 6 men) were briefed, and the RECOs, leaders and service providers of the AS in Lwemba and Babombi (Mandima) were trained; the posting of messages and the distribution of leaflets continued in the health zones of Bunia, Aru, Mongbwalu, Mambasa, Fataki and Mandima. In Haut-Uélé, 24 teachers from Collège La Victoire (including 4 women) were briefed on the PSEA (Public Health Support Program) and the reporting procedure using the toll-free numbers 495555 and 151 in Isiro. Awareness campaigns were also conducted with 279 people in Wamba (AS Maman Kahenga), 54 people and 12 taxi drivers in Isiro, 11 members of PRESICODEV and 7 members of the management team in Boma Mangbetu, 10 nurses from the Gombari General Referral Hospital, and 10 service providers from the Abiangama and Etakamukongo health centers in Pawa, who signed the code of conduct.

- **In North Kivu,** 28 data entry agents were briefed on the PSEA/HS in Beni, where 47 community health workers (members of the ECUMR teams) and 487 community members were sensitized; 21 people (17 women and 4 girls) were sensitized in Butembo and 133 in Katwa (AS Irangya and Masiki), and 19 members of the epidemic management committee were trained in Walikale. Nationally, 749 people were sensitized and 52 providers were trained or briefed during the day, bringing the total to 69,349 people sensitized.

2.13.2. Challenges

- At the national level, 3 out of 7 health districts (42.9%), Ituri, North Kivu, and Haut-Uélé, reported their PSE activities to the national coordination, with only one support partner reporting out of 14 (7.1%; IMC); coverage of affected areas remains low (56 health zones out of 151 with PSE activities, or 37.1%), visibility tools are insufficient, and the effectiveness of community health centers is low. The proportion of health zones aware of the reporting mechanism remains low in Ituri (36.1%; 16/36), Haut-Uélé (46.2%; 6/13), and North Kivu (17.6%; 6/34); In Ituri, the mapping of stakeholders is not controlled and some partners refuse to align themselves; in Haut-Uélé, no partner reports its activities and the pillar lacks means of transport, computer kits and awareness-raising materials; and in Tshopo, focal points lack collection tools and means of mobility.

MESSAGES CLES

ÿ In the past 24 hours, 529 out of 530 suspected cases (96.8%) have been investigated in the provinces that submitted their data.
However, 16 suspected cases remain uninvestigated in Ituri. Furthermore, the contact tracing rate has improved, rising from 79.2% the previous day to 79.1%, but remains below the required threshold of 95.0%. This situation necessitates strengthening epidemiological surveillance and contact tracing to limit the risk of community transmission.

Maintain a level of vigilance regarding epidemiological surveillance aspects, consolidate IPC and CREC actions at the community level while providing quality care in dedicated care structures with adequate reception capacities.

TIMELINE OF KEY EVENTS

ÿ From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

ÿ Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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